

Recognising Care Work(ers)

A Feminist Legal Examination of ASHAs

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Despite being key “frontline” workers, made even more visible by the pandemic, accredited social health activists are not categorised as workers, denying them many entitlements and social security benefits. This paper reviews legal judgments and public policy discourses, locating them in the interstices of women’s unpaid care work and the law, to interrogate the conceptualisation of the ASHA. The terrain of legal recognition has been uneven for ASHAs, with the journey beginning from volunteer to a proposed “worker.” However, many of their demands remain unfulfilled, for which this paper proposes a feminist re-imagination of care work.

In 2022, the World Health Organization named ASHA (accredited social health activist) workers as one of the six recipients of the Global Health Leaders Award, in light of their contribution to global health (Express News Service 2022). This was an acknowledgement post-pandemic where ASHA workers had been at the forefront of the Indian state’s COVID-19 mitigation strategy. The term used for them was front-line workers. The word “frontline” has two meanings—first, it is the part of the army, that is, closest to the enemy and second, the most important or influential position in a debate or movement. In the context of the pandemic, it is clearly those set of people who have been involved most directly in providing some kind of medical service during this public health crisis, thereby being closest to the virus. That number was approximately 30 million, including doctors, paramedics and “other” healthcare service providers. The ASHA workers fall under that other, sitting at the “edge” of the frontline, risking lives and being denied both entitlements and empathy, despite WHO acknowledgement post-pandemic. This paper examines the case of the ASHA workers, from the lens of legal judgments and public policy discourses, to understand the intersections of women’s work, care and law. It begins by delineating the imagination of the state in the creation of an ASHA worker and their lived realities. This is followed by a critical understanding of the state’s rationale by examining the literature on women’s work and care work. Finally, by discussing some recent judgments related to ASHA workers, the paper analyses how, in this case, the law is the site of struggle and change. The conclusion proposes a feminist transformative re-imagination of care work and care worker.

The ASHA ‘Worker’

As per the state’s conceptualisation, ASHA forms a crucial component of the National Rural Health Mission, acting as a link between the public health system and the community. The mission began in 2005, with full implementation targeted by 2012. There are over 10 lakh ASHA workers spread across the country. It is mandated that the ASHA should be a woman who is a resident of the village, between the ages of 25 and 45 years, and should be married, divorced or widowed (Ministry of Health and Family Welfare 2008). The assumption behind this criterion seems to be that unmarried women would move out of their natal village after marriage (Ved et al 2019). However, given the gendered nature of public policy and paradigm on care work, we may also locate this criterion in Palriwala and Neetha’s (2011) gendered familialism, wherein care work

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is seen to belong to the “private” domain, to be handled by women in the household. In the long list of duties of the ASHAs, the guideline stipulates that she is to provide guidance to women on various aspects of maternal and child health, including, preparing for childbirth, safe delivery practices, breastfeeding and complementary feeding, vaccinations and other related aspects of family planning and reproductive health (Ministry of Health and Family Welfare 2008).

The compensation for ASHAs consists of a fixed monthly honorarium from the union and state governments, along with incentives for specific duties, therefore, the total remuneration they receive varies across states. For example, in Telangana, ASHAs are paid ₹6,000, while in West Bengal and Rajasthan they are paid a fixed minimum salary of ₹3,000 (Sinha et al 2021; Ambast 2021). The work that they do also requires a certain set of knowledge, skills and training. Yet, they are not considered workers and therefore paid scanty honorariums. ASHAs are a part of a larger set of “scheme-based” workers, who despite performing a number of functions similar to full-time permanent public employees are considered as voluntary workers and therefore paid an honorarium (Dandamudi and Sreeram 2022). The healthcare workers, in reality, work for more than eight hours each day (Sinha et al 2021). They are subjected to informal work conditions in spite of providing highly specialised care work. As detailed in a survey carried out by the People's Archive of Rural India (PARI), the work done by ASHAs is extensive, with a total of 12 primary tasks and over 60 sub-tasks, spanning information dissemination, monitoring treatment and documentation of health indicators. However, as pointed out by an ASHA worker in the survey, the work that they are actually trained for, which is monitoring and improving maternal and neonatal health indicators, gets sidelined in the process (Bhonsle and Prasad 2020).

One of the descriptions of the ASHA shows how the state conceptualises her role as someone who is “Empowered with knowledge and a drug-kit to deliver first-contact healthcare, every ASHA is expected to be a fountainhead of community participation in public health programmes in her village” (Ministry of Health and Family Welfare 2008). While, during the COVID-19 pandemic, ASHA workers were at the forefront of the Indian state's mitigation strategy, yet they struggled to get access to personal protective equipment (PPE) kits, resources and other essential safety tools. They were the first point of contact for door-to-door surveys, while simultaneously helping out households in quarantine, carrying out awareness campaigns, among many other activities (Sen and Rajeev 2023). In many instances, they even had to borrow or buy smartphones to execute their duties (Brar 2020).

As per the guidelines laid out by the National Health Mission, a host of factors have been deemed critical to the success of ASHAs. These factors include selection of ASHAs by prescribed process, identification of transport for referral of cases from village to facility, timely payment of incentives, and timely replenishment of kits, among others (Ministry of Health and Family Welfare 2008). In contrast to this are the actual experiences of ASHA workers. In a survey conducted by Rao and Chowdhury (2021) during the pandemic, 86% of ASHA workers (total number 201)

interviewed across 10 states reported a fall in incomes since March 2020. It was also observed that 31% of these workers had not received the incentives for the pandemic-related work they had done either. The process is made even more challenging by bureaucratic hurdles, complicated documentation for claims and negligence by supervisors. Moreover, it has now been sufficiently documented that they have not been paid even the meagre honorarium, an issue that persisted even in the pre-pandemic times (Sinha et al 2021; Rao and Chowdhury 2021). For instance, in a study conducted in 2016 it was found that not only had 35% of workers not received their previous month's salary but 40% of them were also bearing the expenses of sustaining the activities in the health centres (Sinha and Bhatta 2019). Earlier, they did get certain incentives when they carried out tasks like enabling certain vaccinations necessary for children, but due to the COVID-19 outbreak, they were totally engaged in pandemic-related activities. Therefore, their earnings were even lower than the pre-pandemic levels.

The work done by ASHAs was crucial in mitigating the pandemic, with the health ministry even praising them for their active participation in COVID-19 household surveys and for screening interstate passengers, migrant workers, and others in the community. Despite this awareness of their vital role in supporting public health, the measures taken to redress their issues remain scant. Owing to the lack of protective gear, many were infected with COVID-19 and some even succumbed to the virus. There were also many instances of ASHAs facing stigma, as they were seen as carriers of the disease and this was coupled with instances of casteism (Krishnan 2021).

An examination of official guidelines reveals a preliminary recognition of the fact that ASHAs require sufficient institutional support. However, the guidelines identify the sources of this support as women's committees, self-help groups, and other health workers like auxiliary nurse midwives (ANMs) and anganwadi workers (Ministry of Health and Family Welfare 2008). It is pertinent to note here how the activities that ASHAs carry out not only subsidise the activities of social reproduction that are essential (Sinha and Rajeev 2023) but also that the state deems institutional support for the ASHAs must come from women themselves, thereby concretising the sexual division of labour that manifests in care work. Moreover, these other women workers are also in equally precarious work engagements and are mostly from the marginalised caste backgrounds. It has been argued that the feminisation of the work in state-sponsored schemes, along with its casual and informal nature, are examples of “care extractivism” (Wichterich 2021). This also assumes significance since in the 2024 interim budget, it was stated that ASHA and anganwadi workers would be given healthcare coverage under the Ayushman Bharat scheme (Press Trust of India 2024). However, their demands related to working conditions, wages, and recognition as workers remain unaddressed.

Care as Work: Feminist Perspectives

Intrinsically linked to the question of women and care, is the notion of work; more specifically the different forms of paid and unpaid work women engage in, and how they influence

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each other. Folbre and England (1999) defined “caring work” as any kind of (paid) work which involves direct, one-to-one interaction with the recipient, like teaching, nursing, therapy, and so on. They argue that the undervaluation of these forms of work relative to the skills they require, known as the care penalty, is a result of the cultural sexism that militates against the recognition of the true value of this work (since these activities are usually associated with women). Care work is also distinct from other forms of work, since it takes a more relational form, wherein caring “activities” are entangled with caring “feelings,” affect, and altruism (Folbre and Nelson 2000).

Studies show that the burden of vital tasks necessary for social reproduction, in a capitalistic system, is predominantly shouldered by women and marginalised groups (Bhattacharya 2017). Some scholars rightly point out that care work, which is the “concrete work” within these activities of social reproduction, is needed by all humans and holds together the whole economy (John and Wichterich 2023). These activities contribute to a common good that benefits society at large, yet they are often linked to notions of altruism and affection, excluding them from considerations of compensation. This was visible in the language used for the healthcare workers and their work during the pandemic, wherein there has been a repeated, intense focus on highlighting their heroism and altruism, without addressing or even ensuring any substantive change in their material conditions. The notions of altruism actually obfuscate power dynamics and justify unfair relations of work. The unpaid care work is termed “unproductive” but at the same time it subsidises the economy (Ghosh 2012). As noted by Oxfam, this “invisible” work contributes at least \$10.8 trillion a year to the global economy (Mercado et al 2020).

The care workers are overworked and underpaid, with abysmal social or legal protection status. The gap in hourly wages of care workers compared to workers in other sectors, termed as the “care pay penalty” is anywhere between 4% and 40% (Oxfam International 2020). They are often not considered to be a priority by the state despite shouldering a large burden of work in the healthcare system. In the case of ASHA workers, the routine and extremely demanding work that they do leaves them depleted. Between 2005–06 and 2015–16, the coverage of four or more antenatal care visits increased from 37% to 51%, and institutional deliveries increased from 39% to 79% (IIPS and ICF 2017). It was also evident from the PARI survey that along with their regular work the amount of paperwork that ASHAs are expected to do keeps piling up and they end up filling more and more surveys.

Palriwala and Neetha (2011) have argued that the paradigm of care in India is based on “gendered familism,” designating care as familial activity to be shouldered by females, and deigning it unproductive. This is also linked to the situation of ASHAs as they are undertaking activities of care which in many ways are extensions of unpaid care work that women are expected to carry out at home (Sinha and Rajeev 2023). Such forms of paid–underpaid care work are also underpinned by caste-class intersectionalities as often care workers belong to

marginalised caste and lower-class locations creating interlocking structures of oppression (Ray 2023; Wichterich 2021). Therefore, in the context of women healthcare workers, a redefinition of the contours of work, more specifically, care work is elemental in transforming women’s bargaining power, agency and thereby, their everyday lives. A powerful means to do that is through legal interventions.

Volunteers Not Workers ?

It is important to discuss the court interventions with respect to ASHA workers and understand whether the role of the judiciary has been restrictive or facilitative in defining the ASHAs. According to the National Health Services Resource Centre (2011), most ASHAs come from poor households and the proportion of ASHAs who are Scheduled Castes (scs) or Scheduled Tribes (sts) is equal to or more than the proportion of the sc or st population in most states. It was emphasised that four activities primarily form ASHAs’ activities—home visits, attending immunisation sessions, regular visit to health facility for training and holding village-level meetings. The report states that more support is needed to enable ASHA workers to reach all houses in her assigned village, overcoming social barriers (National Health Services Resource Centre 2011).

Despite this, the delegitimisation of scheme-based workers started early in the legal space. One such blow was in 2006, when in the *State of Karnataka and Ors v Ameerbi and Ors* (2006) it was held that anganwadi workers engaged under the Integrated Child Development Services (ICDS) programme were not entitled to minimum wages as per the Minimum Wages Act (Singh 2021). According to the court judgment, the ICDS did not constitute an industry, nor did anganwadi workers constitute industrial workmen. While the state may be mandated to pay minimum wages irrespective of financial constraints, there is nothing in law to suggest that it needs to extend to project-based workers. The workers of the latter category cannot come under the mandate of minimum wages, living wages or fair wages. Scheme workers perform a wide array of functions, like those in the health sector or as educators. Some of them work to provide basic nutrition to pregnant women, infants and toddlers. Despite this diversity of work, there are two similarities in innocuousness and debilitating conditions of work. The similarity is that they are all involved in the implementation of schemes, however they all face the abysmal working conditions, marked by long hours, low honorariums, and lack of social security. The scheme workers are also not covered under various labour laws, especially the Industrial Disputes Act (Dandamudi and Sreeram 2022).

In this context, Ramanathan and Chakravarty (2021) talk about the deeply patriarchal state–capital–community nexus that ASHAs work within. They argue that the ASHAs are under a constant pressure at a personal, community and system level. Balancing domestic and professional responsibilities, ensuring that the beneficiaries access services meant for them, and finally the responsibility to meet the system’s desired output, all need to be worked out by ASHAs. By confining the

status of ASHA as a volunteer, the state extracts her labour and, in the process, also ensures the dual purpose of extending health services to the bottom-most level at the least cost. The volunteer status of ASHAs both naturalises care as a gendered practice and institutionalises its devaluation. In a study conducted in Punjab, Marwah (2021) demonstrates how ASHAs, from poor SC or OBC communities with limited education, also had dire compulsions to earn a living. It is their intersectional vulnerability as well as their husbands' unemployment and/or addiction that forced them to underpaid work. It is equally important to remember that ASHAs enabled rural women in Chhattisgarh to gain knowledge beyond the village, created an increased sense of empowerment and they could garner protest against alcoholism and take action against gender-based violence in the state. Overnight rest-rooms and discussions on sexual harassment in monthly staff meetings have been prioritised by the National Rural Health Mission (NRHM) for the safety of the ASHAs (Ved et al 2019).

The 45th session of the Indian Labour Conference (ILC), held in 2013, had recommended that all scheme workers should be recognised as “workers” and not as “volunteers” or “honorary workers” (Ministry of Labour and Employment 2013). There was also an emphasis to include them under the health and life insurance schemes and social security measures (Singh 2021). But the action taken report presented by the government at the 46th meeting of the Standing Labour Committee 2015, rejected each and every one of these demands, except that of the insurance scheme coverage. The report stated that ASHAs are honorary workers under NRHM who only receive performance-based incentives and hence the broader government department was unable to comply with the recommendations of the ILC (Ministry of Labour and Employment 2015). The Standing Committee on Labour 2019–20 recommended that the Code on Social Security include anganwadi and ASHA workers in its definition of “employee.” The 2020 Labour Code, which aims to extend social security to all employees and workers either in the organised or unorganised sectors, excludes ASHA and anganwadi workers (Lok Sabha Secretariat 2020).

The ASHA Workers (Regularisation of Service and Other Benefits) Bill, 2020, affirmed that ASHA workers are one of the main links between government and the general public (Premachandran 2020). They help government in effective implementation of health programmes and considering the importance of their work, it is highly necessary to protect their services and welfare. Section 3 of the proposed bill suggests regularising and conferment of Group C employee status to ASHA workers serving in National Health Mission. Additionally, the need for guaranteed tenure, terms and conditions of service, including remuneration, leave, provident fund, retirement and other terminal benefits was proposed (Premachandran 2020). The hugely deficient bill, not taking into consideration the intersectional vulnerable realities of the ASHA workers, neither specifying from which date (retrospectively) ASHAs would be considered as Group C employees, nor did it see the light of the day.

In October 2018, the Cabinet Committee on Economic Affairs gave ex post facto approval to the ASHA benefit package to include Pradhan Mantri Jeevan Jyoti Bima Yojana and Pradhan Mantri Suraksha Bima Yojana, for those meeting eligibility criteria. In addition, an increase in incentives from ₹1,000 per month to ₹2,000 per month (Press Information Bureau 2018).

In *C H Sharma and 7 Others v State of Maharashtra and 2 Others* (2020), one of the earliest COVID-19-related judgments was delivered on 30 April 2020 by the Bombay High Court. The Nagpur Municipal Corporation had submitted a proposal to provide ₹200 per day, in addition to ₹1,000 per month, to ASHAs. However, they expressed their concern that it would be burdensome on the municipal finances. They categorically stated ASHAs did not have any legal provision making them entitled to food or tea, further suggesting that they could get their own refreshments. The application was filed on behalf of the Nagpur Municipal Corporation Employees Union wherein they brought to the notice of the court the pathetic condition of the ASHAs. The union complained that appropriate remuneration was not being paid to the ASHA workers and the basic safety gears were not provided, in addition to the lack of regular access to even basic refreshments, tea or water. Justice Manish Pitale had recorded the unfortunate treatment meted out by the Nagpur Municipal Corporation to ASHA workers who were performing door to door surveys and at the forefront of the war against COVID-19 (Singh 2021).

Rights of Public Health Workers

On 27 April 2020, the Supreme Court heard a petition seeking provision for PPE for health workers, including doctors, nurses, ward boys, and medical and paramedical professionals, who were working in non-COVID-19 treatment areas since the deadly infection could be contracted from asymptomatic patients. In *Jerry Banait v Union of India* (2020), a three-judge bench, comprising of Justices N V Ramana, Sanjay Kishan Kaul and B R Gavai, had directed the government to examine this issue and make necessary suggestions in the rational use of PPE guidelines so that the PPEs are provided to all health officials working in non-COVID-19 treatment areas. The hierarchy among healthcare personnel acutely became evident through the pandemic even with respect to easier access to PPE kits. By November 2020, the Supreme Court gave orders for reopening of anganwadi centres outside the containment zone. This came after a complaint about the lack of access to proper nutrition by pregnant and lactating mothers. Although the women and child development ministry had noted earlier that the provision of certain essential services like growth monitoring referral, supplementary nutrition for these women were necessary, yet it did not yield any positive outcome. The Court stated that in view of the COVID-19 pandemic globally, it is imperative to ensure that the beneficiaries neither suffer nor fail to access anganwadi services (Hindu 2020). While this was an important decision of the Court, it directly affected ASHA and anganwadi workers, as their work had already increased due to the pandemic.

While some judgments were sensitive to the working conditions of the ASHAs in the context of the pandemic, there were

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opposite trends as well. There were also various protests by ASHA workers across the country during this time, demanding their basic entitlements, and yet most of these protests were met with apathetic silence from the authorities. In some instances, police cases were registered against them for violation of social distancing norms, while others were threatened with termination, as per certain clauses under the epidemic diseases act (Tiwarei 2020). The issue of workers being paid a meagre honorarium than they were entitled to, was an issue even before the pandemic. While awards like the one conferred by the WHO give some recognition, yet the demand to be recognised as full workers (and not volunteers) remains unfulfilled.

The withdrawal of the state from public healthcare as well as its cost-cutting measures, built on the voluntarism of community health workers (John and Wichterich 2023), gets embedded in their everyday work. This was reflected in a study based on the interactions with the ASHAs, where one of the workers expressed her resentment over filing of all work-related information online and taking pictures of the health meetings in the community, when not every worker had access to the smart phones (Pathak 2022).

While “community service” has been the driving force for women to become ASHAs, it is imperative that working conditions and task-based incentives be standardised, upholding the principle of equal pay for equal work as expounded by the Supreme Court—which is thereby “declared law” as per Article 141 of the Constitution (Saidha 2021). This will also be a step towards fulfilling India’s international obligations, particularly under Article 7 of the International Covenant on Economic, Social and Cultural Rights 1966, exhorting the state to ensure just and favourable working conditions for all (United Nations Human Rights 1966). While the Court judgments over the years have not been consistent, both feminist perspectives on care and the jurisprudence around work and worker suggest transformations in the scheme. The struggles by ASHAs have laid bare not only the blatant neglect of their

needs and rights, but also the invaluable work that they do, which is systematically invisibilised.

Conclusions

A re-imagining of care work and care workers is required wherein we move beyond the binary of work and duty/altruism, which is intricately linked to the sexual division of labour as well as a patriarchal response by the state to any kind of care-based services. The articulations of the rights of ASHA workers must take into account the experiences of their everyday lives, vulnerabilities, stigmatisation, and resistance. The paper therefore proposes feminist reflections in imagining and reading care in legal and policy frameworks.

The healthcare workers in the public health system are intricately interconnected with the system and the community that they serve. Thus, their work needs to be defined holistically, which for ASHAs, in the policy terms implies: recognition as workers, a fixed salary,¹ maternity benefits, proper working conditions, and affordable working women’s hostels. These are some of the entitlements and comprehensive support systems that need to be ensured for a dignified existence of a health worker.

ASHA workers are portrayed as focused, skilled, caring, selfless, exhausted, and under-resourced. Rarely, however, are they reported as policymakers and public decision-makers (Enloe 2021). Hence, it is necessary to democratise the public health care system through principles of people-centredness, cooperation, pragmatism, transparency, and inclusivity. It is important to recognise people as equal partners in a public health system, rather than only being paternalistic or authoritative towards them. This can be enabled by foregrounding the voices from all sections of the public health workers, for inclusive decision-making in service delivery and crisis management. Pandemic resilience came from social capital and the willingness of society to come together for the common good, and that can happen when there is shared decision-making and trust in leadership.

NOTE

- 1 In recent protests, ASHAs had demanded that they be paid at least ₹24,000 per month, or have a fixed salary of ₹15,000, with additional incentives (Tiwarei 2022). Essentially, at present, their remuneration is not even comparable to the minimum wage of a skilled worker.

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